

Scope Management Plan

HealthFirst Care – Patient Experience & Operational Efficiency Initiative

This Scope Management Plan defines the boundaries of the HealthFirst Care initiative and breaks the project into a Work Breakdown Structure (WBS) with key milestones. It builds directly on the objectives, functional/non-functional requirements, and constraints established in the Business Requirements Document (BRD) and Requirements Traceability Matrix (RTM) from the prior labs.

1. Project Scope Statement

1.1 Project Objectives

- OBJ-1: Reduce average patient wait times by 20% within 12 months of go-live.
- OBJ-2: Reduce double-booking and scheduling-conflict incidents.
- OBJ-3: Improve inter-departmental communication and handoff efficiency.
- OBJ-4: Improve overall patient satisfaction, particularly in General Medicine and Paediatrics.
- OBJ-5: Consolidate scheduling and record-management data to eliminate data silos, within the approved budget.

1.2 Key Deliverables

- Business analysis artifacts: Business Requirements Document (BRD), Requirements Traceability Matrix (RTM), Stakeholder Analysis & Engagement Plan, and this Scope Management Plan.
- Current-state (As-Is) and future-state (To-Be) process models for appointment scheduling, patient notifications, resource allocation, and inter-departmental handoffs.
- An integrated online scheduling module with real-time conflict detection (FR1), automated patient notifications (FR2), and real-time resource visibility (FR3).
- Integration between the scheduling module and the existing patient record system (FR5), removing data silos flagged by IT.
- A reporting dashboard tracking wait times, resource utilization, and patient satisfaction against the objectives above (FR6).
- Trained administrative, clinical, and IT staff, and a completed phased go-live.
- A final project report and presentation to Hospital Leadership summarizing outcomes against the objectives.

1.3 In-Scope

- Automated online appointment scheduling with real-time doctor/resource availability, to reduce double bookings and scheduling conflicts.
- Automated notifications (email/SMS) for appointment confirmations, cancellations, and delays.
- Integration between the scheduling system and existing patient record systems.
- Dashboards/reporting on resource utilization, wait times, and patient satisfaction.
- Improved inter-departmental handoff and referral workflows (e.g., lab-to-clinician, physiotherapy-to-orthopaedics).
- Process analysis and redesign (As-Is/To-Be models) for the workflows above.

1.4 Out-of-Scope / Exclusions

- Hospital construction or facility expansion projects (e.g., new operating rooms, additional beds).
- Replacement of the core Hospital Information System (HIS) or billing platform — only integration, not replacement, is in scope.

- Hiring or staffing-level changes to nursing/medical rosters (a separate HR initiative).
- Telemedicine or virtual-consultation capability.
- Non-HealthFirst Care partner clinics or third-party referral networks.

1.5 Constraints & Assumptions

Constraints:

- Budget limitations for system upgrades restrict the project to integration with, rather than replacement of, existing core systems.
- The solution must work within existing hospital network infrastructure, which IT reports experiences frequent downtime.
- Implementation must avoid disrupting ongoing patient care, requiring a phased rollout rather than a single cutover.
- Timeline and staff availability for training may be limited given existing staffing pressures.

Assumptions:

- The case-provided datasets and stakeholder profiles are representative of recurring operational patterns, even though each is a small sample.
- Existing IT infrastructure can support integration without a full core-system replacement.
- Clinical and administrative staff will be available for UAT and training ahead of go-live.

2. Work Breakdown Structure (WBS)

The WBS below decomposes the project into nine phases, each ending in a milestone. Where a task implements a specific BRD requirement, the requirement ID is noted in the last column for traceability back to the BRD and RTM.

WBS ID	Task / Deliverable	Type	Related Requirement / Stakeholder
1.0	Project Initiation & Governance	Phase	
1.1	Define project charter and objectives	Task	Aligned to BRD Business Objectives OBJ-1–OBJ-5
1.2	Confirm project sponsor and steering committee	Task	Hospital Leadership
1.3	Approve project budget and constraints	Task	Hospital Leadership; Project Sponsor
M1	Milestone: Project Charter & Governance Approved	Milestone	
2.0	Stakeholder & Requirements Analysis	Phase	
2.1	Conduct stakeholder identification and profiling	Task	Stakeholder Profiles document
2.2	Develop Stakeholder Analysis & Engagement Plan	Task	Completed — prior lab
2.3	Gather and document business requirements	Task	Business Requirements Document (BRD)
2.4	Develop Requirements Traceability Matrix (RTM)	Task	Completed — prior lab
2.5	Prioritize requirements using MoSCoW	Task	Completed — prior lab
M2	Milestone: BRD, RTM & Stakeholder Plan Approved	Milestone	
3.0	Scope & Planning	Phase	
3.1	Define project scope statement	Task	This document

3.2	Develop Work Breakdown Structure	Task	This document
3.3	Develop project schedule and resource plan	Task	
M3	Milestone: Scope Management Plan Approved	Milestone	
4.0	Process Analysis & Redesign	Phase	
4.1	Map current-state (As-Is) scheduling, handoff, and records processes	Task	Stakeholder Profiles pain points
4.2	Analyze appointment, feedback, and resource data for root causes	Task	appointment_data.csv; feedback_data.csv; resource_data.csv
4.3	Design future-state (To-Be) process models for scheduling, notifications, and handoffs	Task	FR1, FR2, FR4
4.4	Validate process models with clinical and administrative stakeholders	Task	Doctors; Nurses; Administrative Staff
M4	Milestone: Process Models Completed & Validated	Milestone	
5.0	Solution Design & Development	Phase	
5.1	Design and configure online scheduling module with conflict detection	Task	FR1
5.2	Build automated patient notification feature (email/SMS)	Task	FR2
5.3	Build real-time resource visibility module	Task	FR3
5.4	Build inter-departmental handoff/referral workflow	Task	FR4
5.5	Integrate scheduling module with existing patient record system	Task	FR5
5.6	Implement network reliability and security remediation	Task	NFR2, NFR3
M5	Milestone: Core Solution Build Complete	Milestone	
6.0	Reporting & Dashboard Development	Phase	
6.1	Define KPIs for wait time, resource utilization, and satisfaction	Task	FR6
6.2	Design and build the reporting dashboard	Task	FR6
6.3	Validate dashboard outputs against BRD/RTM objectives	Task	OBJ-1, OBJ-4, OBJ-5
M6	Milestone: Dashboard Completed	Milestone	
7.0	Testing & Validation	Phase	
7.1	Conduct system and integration testing	Task	
7.2	Conduct user acceptance testing (UAT) with staff	Task	Administrative Staff; Doctors; Nurses
7.3	Resolve defects and retest	Task	
M7	Milestone: UAT Sign-off	Milestone	
8.0	Deployment & Training	Phase	
8.1	Phase 1 rollout: Scheduling & Notifications MVP	Task	FR1, FR2, NFR1, NFR2, NFR3

8.2	Phase 2 rollout: Resource Visibility & Records Integration	Task	FR3, FR5, NFR4
8.3	Phase 3 rollout: Reporting & Handoff Workflow	Task	FR4, FR6
8.4	Train administrative, clinical, and IT staff	Task	
M8	Milestone: Phased Go-Live Complete	Milestone	
9.0	Project Closure & Final Presentation	Phase	
9.1	Consolidate results against business objectives	Task	OBJ-1–OBJ-5
9.2	Prepare final project report and lessons learned	Task	
9.3	Deliver final presentation to Hospital Leadership/Sponsor	Task	Hospital Leadership
M9	Milestone: Final Presentation Delivered & Project Closed	Milestone	

3. Milestone Summary	
Milestone	Marks Completion Of
M1	Project Charter & Governance Approved
M2	BRD, RTM & Stakeholder Plan Approved
M3	Scope Management Plan Approved
M4	Process Models Completed & Validated
M5	Core Solution Build Complete
M6	Dashboard Completed
M7	UAT Sign-off
M8	Phased Go-Live Complete
M9	Final Presentation Delivered & Project Closed